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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°154 /MVE-BDBV/15/10/2026

Report date: October 15, 2026 Publication date: October 16, 2026

HIGHLIGHTS (last 24 hours)

- **Seventy-one (71) new confirmed cases of Ebola Virus Disease (EVD), including 18 community deaths, have been reported. These cases originated from the provinces of Ituri (41 cases), North Kivi (22 cases), Tshopo (7) and Haut-Uele (1). In addition, 14 deaths were recorded in the CTEs.**
- **A total of 462 new samples were analyzed; 71 of these samples tested positive for the virus. Bundibugyo, representing a 15.4% positivity rate**

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9348	4 510	48,2 %	916	2 507	79,2%

Affected provinces and affected health zones

7 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 Health Zones affected out of 167 (38,9%)

Summary of key points from the last 24 hours

In the last 24 hours, 71 new confirmed cases of viral disease Ebola (EVD) has been reported in four provinces: **Ituri (41 cases), North Kivu (22 cases), Tshopo (7 cases) and Haut-Uélé (1 case)**

In addition, **14** deaths were recorded in the CTEs (6 in Ituri, 6 in North Kivu, and in Haut Uélé 2, bringing the total number of confirmed deaths for the period in question to 32.

In addition, **21 patients (11 in Ituri and 10 in North Kivu)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has accumulated **9,348 confirmed cases and 4,510 deaths,** representing a case fatality rate of **48.2** %. Ituri remains the epicenter of the epidemic, accounting for **73.2% of cumulative confirmed cases and 57.7% of new confirmed cases.** reported in the last 24 hours.

65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 15, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	41	6847	3 164	46,2%	28/36 (77,8 %)
North Kivu	22	2013	1 146	56,9%	17/34 (50,0 %)
Haut-Uélé	1	399	169	42,4%	7/13 (53,8 %)
Boss	7	71	25	35,2%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**71**	**9 348**	**4 510**	**48,2%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 15, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6847**	**3164**	**46,2%**	**41**	**10**	**6**	**16**
Give	11	1	9,1%				0
Understand	9	6	66,7%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1869	623	33,3%	3	1	1	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	187	113	60,4%	2	0	1	1
Liter	307	155	50,5%				0
Logo	14	8	57,1%				0
Fight	36	9	25,0%				0
distributed	4	1	25,0%				0
Washing	65	25	38,5%	3	2	0	2
Darkness	298	149	50,0%	21	7	0	7
Mangala	375	162	43,2%	3	0	0	0
Mongbwalu	713	342	48,0%				0
Nanny	329	190	57,8%			2	2

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Nizi	822	297	36,1%			2	2
Legumes	134	39	29,1%				0
Jungle	19	5	26,3%				0
Rwampara	1151	398	34,6%	9	0	0	0
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**2013**	**1146**	**56,9%**	**22**	**7**	**6**	**13**
Me	498	336	67,5%	7	5	5	10
Farm	23	7	30,4%				0
Butembo	311	187	60,1%	4	0	0	0
Ten	1	0	0,0%				0
Kalunguta	75	29	38,7%	1	0	0	0
Cut	637	390	61,2%	4	1	0	1
Source	3	2	66,7%				0
Kyondo	42	21	50,0%	2	0	0	0
Lubero	32	14	43,8%	3	1	0	1
Be careful.	70	28	40,0%	1	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	15	5	33,3%				0
Masereka	39	13	33,3%			1	1
The museums	151	71	47,0%				0
Mutwanga	24	5	20,8%				0
Oicha	47	21	44,7%				0
Vuh	40	14	35,0%				0
Upper Uele	**399**	**169**	**42,4%**	**1**	**0**	**2**	**2**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	105	46	43,8%				0
Lining	102	51	50,0%	1	0	1	1
Beans	5	3	60,0%				0
Ordinary	139	48	34,5%	0	0	1	1
Boss	**71**	**25**	**35,2%**	7	1	0	1
Bafwasende	17	2	11,8%	7	1	0	1
Kabondo	8	1	12,5%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	14	43,8%				0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0

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Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**9348**	**4510**	**48,2%**	**71**	**18**	**14**	**32**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

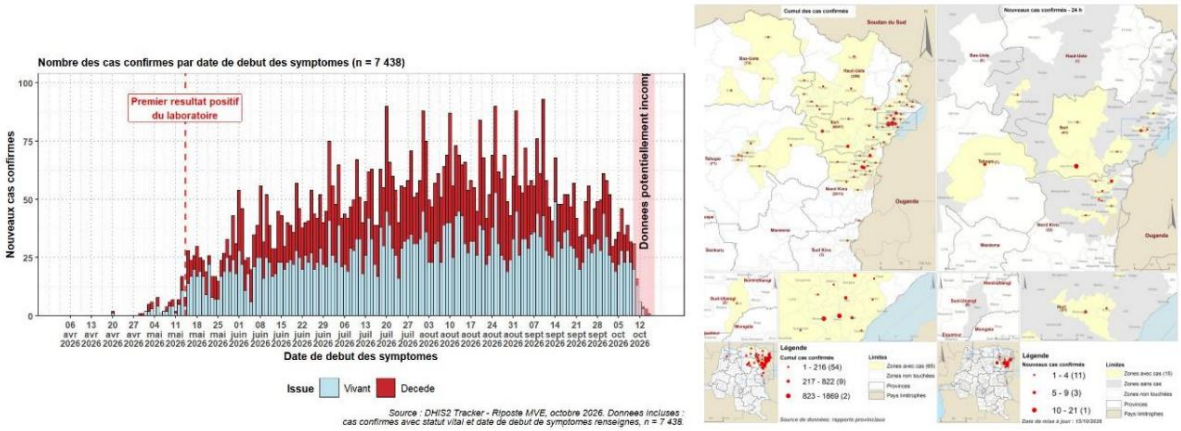
Table 3. Status of alerts notified as of October 15, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	184	0	184	9	0	175	0	9	9
Bas Uélé	245	2	247	64	2	181	0	66	0
Ituri	1 450	73	1 523	170	73	1 280	0	227	137
North Kivu	1 522	64	1 586	147	64	1 375	0	211	94
Sud Ubangi	98	0	98	1	0	97	0	1	0
Boss	139	2	141	13	2	126	0	15	0
Total General	**3 638**	**141**	**3 779**	**404**	**141**	**3 234**	**0**	**529**	**240**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.

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1.COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- On October 15, 2026, 3,779 alerts were reported in the five provinces that submitted their data: North Kivu, Ituri, Bas-Uélé, Haut-Uélé, and Tshopo. Of these, 3,421 (92.9%) were verified: 545 (15.9%) were validated as suspected cases. In total, 528 suspected cases (97.1%) were investigated, and 240 (44.1%) were transferred to COVID-19 treatment centers.

- Of the 35,306 contacts to be followed up in the five provinces that submitted their data, 27,976 were seen in the last 24 hours, representing a follow-up rate of 79.2%, up from the previous day (78.0%) but below the required threshold of 95.0%. This rate was 100.0% (44/44) in Sud-Ubangi, 91.8% (626/682) in Bas-Uélé, 87.9% (1,515/1,723) in Haut-Uélé, and 83.1% (14,204/17,084) in Nord-Kivu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

- On October 15, 14 alerts were notified to the PoE/PoC of the five monitored provinces: 12 in Ituri, 1 in North Kivu and 1 in South Kivu. These include 7 live alerts validated as suspected cases (6 in Ituri and 1 in North Kivu), of which 3 (42.9%) were transferred to CT/CTE, and 4 intercepted bodies (3 in Ituri and 1 in South Kivu), of which 2 (50.0%) were swabed or secured; all live alerts were investigated.

Table 4. Status of key PoE/PoC indicators by province, as of October 15, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,4% (60/61)	90,5% (19/21)	55,6% (5/9)	100,0% (18/18)	90,0% (9/10)	93,3% (111/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Name of people switched to PoE/PoC	146 665	76 319	6 945	18 265	16 881	265 075
% of travelers screened	97,5%	98,0%	98,1%	98,3%	99,0%	97,8%
% of travelers who washed their hands	97,3%	98,0%	98,1%	98,6%	99,0%	97,7%
% of travelers made aware	99,6%	98,0%	100,0%	100,0%	100,0%	99,2%
Number of alerts notified	12	1	1	0	0	14

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Number of live alerts validated (suspected cases)	6	1	0	0	0	7
Number of bodies recovered today	3	0	1	0	0	4
% of suspected cases transferred to the CTE/CT	33,3%	100,0%	THAT	THAT	THAT	42,9%
% of lifeless bodies swabed/secured	66,7%	THAT	0,0%	THAT	THAT	50,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	THAT	THAT	THAT	100,0%

2.2.2. Challenges

- **In Ituri,** insufficient mobility for supervision in the health zones of Rwampara, Nizi, and Bunia, and the delayed payment of service providers for September, persist; the Avakubi Point of Control (PoC) remains non-functional due to insecurity. In North Kivu, the coverage rate is 90.5% (19/21), the Butuhe PoC remains closed since the security incident of September 26, 2026, the security threat persists against service providers along the Butembo axis, alerts from the PoCs are not being addressed in a timely manner, and the PoCs lack consumables, communication equipment, and a dedicated ambulance. In South Kivu, the coverage rate remains at 55.6% (5/9), the Baraka-Fizi health post outposts are not reporting, and the Kavimvira and Kalundu PoEs are not operational.

  SNCC Port and Baraka lack isolation systems. In Haut-Uélé, the Mutowamba PoC failed to transmit data due to network outages. In Bas-Uélé, only 19 out of 41 PoE/PoCs (46.3%) are operational, PNHF services are not functioning, and the poorly trained and demotivated service providers lack water, transportation, and visibility badges.

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 41 new positive results (31 living and 10 dead) out of 270 samples received and analyzed (positivity: 15.2%).

- **North Kivu :** 22 new positive results (15 living and 7 dead) out of 128 samples received and analyzed (positivity: 17.8%).

- **Haut-Uélé :** 1 new positive result (1 live) out of 21 samples received and analyzed (positivity: 4.8%).

- **Tshopo :** 7 new confirmed cases (6 living and 1 death) out of 16 samples received and analyzed (positive: 43,8%).

- Bas Uélé: 27 new samples received at the Buta and Isiro laboratory, all analyzed negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 16 out of 26 rings were opened (61.5%), 28 out of 35 households were decontaminated (80.0%), and 4 out of 6 health facilities were decontaminated (66.7%). Of 102 EDS alerts (including 14 postponements from the previous day), 64 EDS were carried out (62.7%), and 69 out of 90 bodies were swabed (76.7%).

- **Haut-Uélé:** 6 out of 6 planned health facilities (100%) were decontaminated (1 in Isiro, 1 in Pawa, and 4 in Wamba), 69 service providers were briefed (28 in Isiro, 28 in Pawa, 9 in Wamba, and 4 in Boma Mangbetu), and 9 health facilities were assessed, with scores ranging from 13.0% at the Bevesieni Health Center to 80.0% at the Nebobongo Health Center; no rings were opened, nor were any households decontaminated. All 3 emergency health alerts in Wamba resulted in a full emergency health assessment with swabs (3/3).

- **North Kivu:** 17 out of 22 infection control rings were opened (77.3%) and 10 out of 23 households were decontaminated (43.5%); 156 infection control centers (ICCs) were monitored, 325 service providers were briefed, and 14 ICCs were assessed (average IPC score of 49.4%). 15 out of 26 primary healthcare professionals (PHPs) were classified as high risk of exposure, with no PHPs infected among the last 28 confirmed cases. Of 57 emergency health alerts (EHS) and 50 reports from the previous day, 62 EHSs were carried out (57.9%) and 57 bodies were swabed; 12 IPC-WASH kits were distributed to two schools in Butembo on Global Handwashing Day, and 10,000 liters of water were provided to the Katwa treatment center with support from CAUB and UNICEF.

- Bas-Uélé: 9 out of 9 community health workers were monitored in the health zones of Buta, Viadana, and Ganga, and 46 workers and 12 hygiene specialists were briefed on the use of PPE, waste management, and sorting (100%); the medical waste management systems are functional at Buta General Referral Hospital (12/12) but not at Rubi Lisanga Hospital (1/5), and the infection prevention and control (IPC) scores

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They reached 62% at CSR Limba and 18% at CS Mazulu (initial assessment). Two complete DHS, with 2 swabs, were carried out in Ganga (AS Agri Uélé).

2.4.2. Challenges

- **In Ituri,** the opening of the rings remains insufficient (16/26; 61.5%), 24 EDS failed and 14 were postponed to the next day, and 21 bodies out of 90 (23.3%) could not be swabed.

In North **Kivu,** gaps in the opening of infection control rings persist in Katwa, Butembo, and Masereka; only 10 out of 23 households have been decontaminated; no decontaminated household has received kits; 45 health assessments remain postponed until the following day; and 36 of the 57 reported deaths (63.2%) occurred in non-dedicated health facilities. In Tshopo, infection prevention and control (IPC) supplies and materials are lacking in health facilities and communities; there remains an insufficient number of frontline healthcare providers trained in Ebola infection prevention and control (EPC); and the incinerator at the Makiso-Kisangani General Referral Hospital, which also serves the laboratory, is out of service. Only 5 out of 104 health facilities (4.8%) exceed an IPC score of 80%. In Haut-Uélé, only 4 out of 7 affected health zones have submitted their IPC data, and IPC kits are lacking in health facilities. In Bas-Uélé, 16 out of 20 priority ESSs do not have functional triage, 17 out of 20 do not have water storage covering 72 hours and 20 out of 20 do not have mattresses; the low reporting of community death alerts and the low community engagement limit EDS.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 60 new admissions were recorded against 82 discharges, including 11 recoveries (confirmed). 384 patients are hospitalized, representing an occupancy rate of 37.0% (47.0% for suspected cases and 27.0% for confirmed cases), including 44 critical patients; 32 patients were evacuated by ambulance and 369 out of 377 patients (97.9%) received a hot meal.

- **In North Kivu,** 97 new admissions were recorded against 86 discharges, including 10 recoveries (Beni 5, Katwa 3 and Kalunguta 2). Thus, 464 patients (161 confirmed and 303 suspected, including 67 children under 5 years old) are hospitalized: 333 in Ebola Treatment Centers/Treatment Centers (392 beds), representing an occupancy rate of 84.9%, and 131 in non-dedicated Alzheimer's Health Centers; 5 patients.

- **In Haut-Uélé,** 5 new admissions of suspected cases (Wamba 3 and Pawa 2) were recorded, compared to 8 discharges: 6 non-cases in Pawa and 2 deaths of confirmed cases, with no recovery. Thus, 44 patients (16 confirmed and 28 suspected cases, including 7 children under 5 years old) are hospitalized out of 169 beds, representing an occupancy rate of 26.0%, including 4 patients in serious or critical condition; 46 patients, 55 caregivers, and 70 people with disabilities received hot meals and were transported by ambulance.

- **In Tshopo,** 8 new admissions (7 confirmed cases from the Bafwasende Treatment Center and 1 suspected case), as well as the readmission of a recovered PPL patient, were recorded, compared to 1 discharge (non-case), with no deaths at the Treatment Center. Thus, 21 patients (13 confirmed cases, including 9 in Bafwasende, and 8 suspected cases) are hospitalized out of 36 beds, representing an occupancy rate of 58.3% (92.9% for beds for confirmed cases), with no patients in serious condition; 3 confirmed cases remain lost to follow-up (2 in Bafwasende and the motorcyclist from AS Marie Antoinette). Since the beginning of the outbreak, 41 confirmed cases have received experimental treatment and 452 people have received post-exposure prophylaxis.

- **In Bas-Uélé,** no new confirmed cases have been recorded; 3 patients remain in isolation, without recovery
no death reported.

2.5.2. Challenges

- **In Ituri,** the low capacity of some CTE/CT (Nyankunde, Kilo, Mangala and Biakato), the care of 44 critical patients, the blood stock reduced to 16 bags, the lack of food for 8 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (18/33) remain worrying.

- **In North Kivu,** the saturation of suspected beds (175/183; 95.6%), the care of 131 patients (28.2%) outside of CTE/CT, an ambulance response capacity limited to 50.0% (14/28) and the incompleteness of ambulance data (Butembo and Mutwanga) remain worrying.

- **In Haut-Uélé,** communication with the CTE remains difficult due to the lack of a functional Starlink connection in
all sites.

- **In Tshopo,** the lack of resuscitation equipment, the absence of specialists at the CTE and the lack of
Staffing for post-exposure prophylaxis weakens the management of the condition.

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- In Bas-Uélé, the lack of an ambulance for transferring patients and of a standardized CTE, still under construction at the HGR of Buta, as well as the lack of computer kits, remain major operational challenges.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri,** vaccination of PPLs with the Ervebo vaccine continued: 87 additional people were vaccinated, bringing the total to 1,120 vaccinated (460 women and 660 men, including 13 pregnant women and 26 breastfeeding women; 2 MAPI), and research on the effectiveness of the Ervebo vaccine against the BDBV strain continued.
- **In Haut-Uélé,** 503 PPLs were vaccinated out of 1,060 pre-registered (47.5%), including 470 in Isiro and 33 in Pawa.

2.6.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,120 vaccinated) for a provisional target of 1,000).
- In Bas-Uélé, the implementation of the UCCs is not effective, and vaccines are not available in sufficient quantities. sufficient funds for vaccination are still pending.

2.7. Continuity of Care

2.7.1. Main actions

- **In North Kivu,** the baseline assessment at Carl Becker Hospital revealed cramped conditions (a single bed), a lack of PPE, pedal bins, and hand sanitizer, a functional emergency department without a backup generator, 40% of maternity staff not trained in SONUB and ANCr, the absence of a neonatology unit, and the lack of integration of TB and HIV vaccination and care; 16 confirmed cases were recorded there, including 1 staff member, and 5 staff members are in quarantine. Nutritionally, 188 confirmed cases, 195 suspected cases, 585 staff members, and 200 caregivers received hot meals, 50 patient households received food aid, and all 16 separated infants are receiving breast milk substitutes (16/16).

- **In Tshopo,** the Alwaleed Health Center and the Ketele Military Health Center were assessed: at Ketele, 15 out of 20 staff members (75.0%) are vaccinated against Ebola and 28 people living with HIV are receiving follow-up care, but HIV testing supplies are unavailable; at Alwaleed, the triage area and incinerator are operational, and 55 out of 55 children were vaccinated during a community health center visit. The recovery plan for the Massina Health Center is being developed, and follow-up care for recovered patients with complications continues at the Makiso-Kisangani General Referral Hospital.
- **In Ituri,** 74 children received nutritional support from LNPE/SLM: 7 out of 7 at the CTE, 6 out of 6 at the crèche and 61 out of 61 in the community.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 21,237 home visits reached 81,678 people; 481 educational talks brought together 15,287 people, 166 mass awareness campaigns reached 6,945 people, 23 community debates involved 480 people, and 7 advocacy sessions engaged 20 religious authorities and leaders in Rwampara and Komanda. A total of 309 community alerts, including 139 deaths, were reported. 38 out of 48 feedback requests (79.2%) and 6 out of 6 incidents (100%) were resolved within 48 hours, and the CREC facilitated the pre-listing of 184 contacts around 4 confirmed cases, the decontamination of 40 out of 40 households (100%), 3 facilities and 3 social and solidarity economy (SSE) facilities, 46 out of 62 EDS (74.2%), the announcement of 20 out of 20 positive results and the transfer of 17 out of 19 suspected cases (89.5%) to Ebola Treatment Centers (ETCs). In Rwampara.

- **In Bas-Uélé,** 453 home visits reached 1,068 people in the Ganga health zone, 3 mass awareness campaigns reached 603 people (Ganga, Aketi and Titule) and 3 educational talks reached 72 people (Ganga and Viadana); a community dialogue brought together 51 people (21 women and 30 men) in the Sopi health zone (Ganga) with the Bamboule group leader, around the latest confirmed case, and 23 travellers were sensitized at the Dakwa Centre checkpoint.

- **In North Kivu,** 11,120 home visits reached 39,389 people, 129 educational talks reached 3,023 people, 16 mass communications reached 3,265 people and 3 briefings reached 87 people; 263 community alerts were notified out of 650 expected (40.5%), 11 out of 15 feedbacks (73.3%) were resolved within 48 hours and 1,736 out of 4,750 people (36.5%) were engaged in community actions.

  The release of 2 healers from the CTE in Katwa and 3 healers from the CTE in Beni was accompanied by testimonies and awareness-raising in the AS of Malepe and Ngongolio, ECUMR teams from pressure groups and leaders were formed for the resistance hotspots of Eringeti (Oicha) and Wayene (Katwa), U-Reporters targeted the indigenous populations of the displaced persons camps in Oicha and 28 school principals and teachers were briefed in Kalunguta.

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- **In Tshopo,** a motorized caravan traveled through the streets of Kisangani to raise awareness among the population about preventative measures and the reporting of alerts. Home visits continued in the health zones of Mangobo, Tshopo, Kabondo, and Makiso-Kisangani, and advocacy efforts were undertaken with the management of BRALIMA. Approximately 101,223 people were reached and 42 service providers were briefed in Lubunga, while all 78 community alerts and 2 feedback reports were addressed.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri,** 20 out of 31 cases (64.5%), 64 confirmed cases under follow-up out of 122 (52.5%), 20 new suspected cases out of 60 (33.3%), and 114 suspected cases under follow-up out of 176 (64.8%) benefited from SMSPS interventions. The teams supported 146 caregivers and 56 people living with HIV/AIDS (PLHIV), followed up with 17 out of 23 recoveries (73.9%) in the community and 15 out of 15 children in daycare, provided support to 7 out of 11 separated and orphaned families, reintegrated 5 out of 11 recoveries, and announced 34 results, 20 of which were positive; 40 children are currently being monitored.

- **In North Kivu,** 25 of the last 28 confirmed cases (89.3%), 212 confirmed cases under follow-up, 318 new suspected cases, and 215 suspected cases under follow-up received SMSPS interventions in 5 out of 7 health zones that reported cases (71.4%); teams assisted 88 households and 32 households of separated and orphaned children out of 50 (64.0%), supported 67 people living with HIV, monitored 5 out of 5 children in daycare, reintegrated 12 recovered individuals, monitored 7 out of 10 recovered individuals (70.0%) in the community, and announced 137 results, including 28 positive results; 342 people (123 men, 145 women, and 74 children) received psychoeducation with UNICEF support

- **In Haut-Uélé,** only Isiro and Boma Mangbetu reported: 5 confirmed cases under follow-up, 1 new suspect and 4 suspects under follow-up benefited from SMSPS interventions, 16 companions and 27 PPLs were supported and 2 results, negative, were announced in Isiro, where 2 non-cases, including 1 PPL, were discharged from the CTE.

- **In Tshopo,** 4 out of 4 confirmed cases under follow-up (100%), 1 out of 1 new suspected cases, and 5 out of 5 suspected cases under follow-up received SMSPS interventions, but none of the last 7 confirmed cases in Bafwasende have yet received treatment (0/7); 386 people, including 251 students from Tshopo High School, received psychoeducation, 20 people with mental health conditions (PMLs) received support, 3 test results, all negative, were announced, and 3 out of 4 resistances were overcome. Training of community leaders in community mental health was launched.

2.9.2. Challenges

- **In Ituri,** only 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone, which concentrates 51.2% of the province's confirmed cases of the day, is still not covered.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** data from Pawa and Wamba have not been collected, the mobility of APS remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Pawa and Wamba and the pillar does not have computer equipment.

- In Tshopo, the 7 new confirmed cases in Bafwasende are not yet covered, the APS of the Wanie-Rukula ZS still need to be trained and the pillar lacks communication units and motorcycles for its travels.

2.10. Logistics

2.10.1. Main actions.

- Ensure the mobility of all field teams in the affected provinces; in North Kivu, construction work on the cabins at PoC PK5 and the cabin and passenger walkway at Beni airport continues with IOM support; in Haut-Uélé, materials have been packed for the activation of two PoCs (PK18 and Bangwene) in the Wamba health zone; in Bas-Uélé, construction of the CTE at Buta General Referral Hospital (partitioning of rooms and toilets) and rehabilitation of the meeting room and offices of the Provincial Health Division with WHO support continues, surveillance teams have been deployed to Ganga and Viadana and the Buta health zone, the General Referral Hospital and the laboratory have been supplied; in Tshopo, the laboratory received a solar panel kit and a split system from JHPIEGO.

2.11. Security

2.12. Main actions

- **In North Kivu,** the EDS team received security coverage for the burial of a confirmed case at the Bundji public cemetery, and the IPC team for the decontamination of two households in the Mangote health district; the threat persists against providers at the Points of Control (PoCs) along the Butembo axis, and the Butuhe PoC remains closed since the incident of September 26, 2026. In Tshopo, community resistance and physical attacks or threats against rapid response teams were reported in Bafwasende, where the body of a patient

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was forcibly taken from the Red Cross EDS team on 14/10/2026; in Ituri, the Avakubi PoC remains non-functional due to insecurity.

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- **In Ituri,** 11 Rwankole CTE providers (6 women and 5 men) were briefed on PSEA/HS and signed the code of conduct, 15 RECOs and community leaders (9 women and 6 men) were briefed in Mongbwalu, and 608 people were sensitized in the community, including 594 students and teachers from Anviiko and Amen schools in Aru and 14 tricycle drivers in Bunia; the posting of messages and the distribution of leaflets continued in the health zones of Bunia, Aru, Mongbwalu, Mambasa and Fataki. In Haut-Uélé, in the Isiro health zone, 24 caregivers and PPLs (7 men and 17 women) from Telalingbi Hospital, 6 other PPLs and one caregiver, as well as 24 teachers from La Victoire College (20 men and 4 women) were made aware of the PSEA (Public Health and Social Action Program) and the reporting procedure using the toll-free numbers 495555 and 151; the PPLs and 17 teachers signed the code of conduct

In North Kivu, 20 new providers were sensitized on the PSEA and the toll-free number with the support of IMC, as well as 23 ECUMRs (22 men and 1 woman) in Beni with the WHO and 92 young people (67 girls and 25 boys) with the ICRC in the Beni health zone; in the Butembo health zone, 40 people (32 women and 8 men) were sensitized on the PSEA and the toll-free numbers 495555 and 151 with the support of the WHO, and 43 leaflets were distributed.

2.13.2. Challenges

- At the national level, 3 out of 7 health districts (42.9%), Ituri, North Kivu, and Haut-Uélé, reported their PSE activities to the national coordination, with 3 support partners reporting out of 14 (21.4%). Coverage of affected areas remains low (56 health zones out of 151 with PSE activities, or 37.1%), visibility materials are insufficient, and the effectiveness of community health centers is low. The proportion of health zones aware of the reporting mechanism remains low in Ituri (36.1%; 16/36) and North Kivu (17.6%; 6/34). In Haut-Uélé, the pillar lacks transportation, computer equipment, and awareness-raising materials.

MESSAGES CLES

Over the past 24 hours, 529 out of 545 suspected cases (97.1%) have been investigated in the provinces that submitted their data. However, 16 suspected cases remain uninvestigated in Ituri. Furthermore, the contact tracing rate has improved, rising from 78.0% the previous day to 79.3%, but remains below the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.

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Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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